Panitumumab + FOLFIRI

Supportive-care review sheet · Individualise with the clinical team; not a prescription.

Day 1: panitumumab, irinotecan, folinic acid and fluorouracil, followed by a fluorouracil pump for about 46 hours; disconnection is arranged around Day 3. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

Before treatment

Confirm the exact indication, validated extended RAS wild-type result, ECOG and benefit/risk, allergies and lung history. Check FBC, kidney/liver profile and magnesium, calcium and potassium. Confirm DPD testing before fluorouracil. Interpret uracil phenotyping cautiously with renal impairment.

Skin and nails

Use the local pre-emptive EGFR skin-care policy: moisturiser, sun protection, and prescribed topical treatment or antibiotics as indicated. Record severe episode number, doses withheld and recovery before restart; inspect for infection.

Minerals and follow-up

Monitor magnesium, calcium and potassium before and during treatment and for up to eight weeks afterwards. NCCP specifies monthly electrolytes for two months after panitumumab. Replace as indicated; there is no NCCP numerical automatic hold threshold.

Antiemetics and premedication

Chemotherapy backbone: moderate risk; panitumumab: low risk. Use NCCP/local antiemetic guidance and an individual rescue plan. No routine panitumumab premedication; prior reactions may change the plan. NCCP includes atropine 250 micrograms SC; check contraindications, including glaucoma.

Bowel symptoms and hydration

Provide separate early cholinergic-syndrome and delayed-diarrhoea instructions. Later diarrhoea can begin >24 hours after irinotecan. Give the local written antidiarrhoeal and escalation plan; treatment-related diarrhoea must fully resolve before irinotecan.

Administration and access

Panitumumab is given in saline through a 0.22 micron filter and is incompatible with glucose. Confirm irinotecan/folinic-acid administration, then bolus fluorouracil and the 46-hour pump. Arrange professional disconnection and a written pump/spill plan.

Medication and safety review

Do not combine panitumumab with bevacizumab-containing chemotherapy or IFL. Review all medicines and supplements with pharmacy. Also check fluorouracil and irinotecan interactions, cardiac risk and the whole regimen’s reproductive precautions.

Before discharge

Use teach-back for skin care, symptoms requiring same-day or emergency help, contact numbers and the next review. Give the visual guide, treatment passport and SACT card. Eye pain/visual change, new lung symptoms and severe skin infection need prompt assessment.

> Resolve source ambiguities with consultant/pharmacy: organ impairment is broadly excluded on page 2, yet table 3 provides adjustment advice. ANC recovery wording differs between two and four weeks. Baseline ANC <2 or platelets <100 are exclusions; subsequent-cycle limits differ. Do not transplant standalone-backbone rules or calculate a reduction without the current component dose level.

Sources: NCCP 00448 v4b · Vectibix SmPC · NCCP antiemetic guidance

SACTCheck v0.83.5 · Developed by Dr Paul O’Brien · Sources checked 16 October 2026 · Clinical/pharmacy review pending.